

Krisovin

tablet 500mg

DESCRIPTION

A 12 mm diameter, round, convex, scored, marked "UP 500" white tablet. Each tablet contains Griseofulvin 500mg.

INDICATIONS

Griseofulvin is administered by mouth in the treatment of infections caused by various species of Epidermophyton, Microsporum, and Trichophyton. It is generally given for such infections that involve the scalp, hair, nails and skin and which do not respond to topical treatment such as Tinea barbae, Tinea capitis, Tinea corporis, Tinea cruris, Tinea pedis and Tinea unguium. Infections of the soles of the feet, the palms of the hands and the toenails respond slowly.

PHARMACODYNAMICS

Griseofulvin is a fungistatic antibiotic which inhibits fungal cell mitosis by causing disruption of the mitotic spindle structure, thereby arresting the metaphase of cell division. It may also interfere with DNA production. It is deposited in varying concentrations in the keratin precursor cells of skin, hair, and nails, rendering the keratin resistant to fungal invasion. As the keratin is shed, it is replaced with healthy tissue.

PHARMACOKINETICS

Absorption of griseofulvin from the gastro-intestinal tract is variable and incomplete, but is enhanced by reducing the particle size and by administration with a fatty meal. Absorption of micronised griseofulvin, as in Krisovin Tablet, ranges from 25%-70% after an oral dose. Peak serum concentrations occur approximately 4 hours following administration of a single dose of 500 mg of micronised griseofulvin. It is deposited in varying concentrations in the keratin precursor cells and is concentrated in the stratum corneum of the skin, nails and hair, thus preventing fungal invasion of newly formed cells. It can be detected in the stratum corneum of the skin within a few hours following administration. Concentrations of 12-25 mcg/g are maintained in skin during long-term administration, while concurrent plasma concentrations remain at about 1-2 mcg/mL. Only a very small fraction of an oral dose is distributed in the body fluids and tissues. Griseofulvin has an elimination half-life of 9-24 hours, and is metabolised by the liver mainly to 6-methyl-griseofulvin which is excreted in the urine, and its glucuronide conjugate. A large amount (36%) of a dose of griseofulvin of reduced particle size appears unchanged in the faeces; less than 1% is excreted unchanged in the urine; some is excreted in the sweat.

DOSAGE

Adult

Onchomycosis, tinea pedis:
500 mg every 12 hours.
Tinea capitis, tinea corporis, tinea cruris:
250 mg every 12 hours OR 500 mg once daily.

Pediatric

5 mg/kg body weight OR 150 mg/square meter body surface every 12 hours OR 10 mg/kg body weight OR 300 mg/square meter body surface once daily. Children 14-23 kg: 62.5-125 mg every 12 hours OR

125-250 mg once daily. Children >23 kg: 125-250 mg every 12 hours OR 250-500 mg once daily.

SIDE EFFECTS

The most frequent side effect is headache which is usually mild and transient. Confusion, hypersensitivity (skin rash, hives or itching), oral thrush (soreness or irritation of mouth or tongue), photosensitivity (increased sensitivity of skin to sunlight), dizziness, gastrointestinal reactions (diarrhoea, nausea or vomiting, stomach pain), insomnia and unusual tiredness have been reported less frequently. Dry mouth, altered sensation of taste, have also occurred. Rarely, granulocytopenia or leukopenia (sore throat and fever), angioedema, erythema multiforme, toxic epidermal necrolysis, proteinuria, hepatitis (yellow eyes or skin) and peripheral neuritis (numbness, tingling, pain or weakness in hands or feet) have also occurred.

Skin and subcutaneous tissue disorders

Frequency 'not known': Stevens-Johnson syndrome, toxic epidermal necrolysis, drug reaction with eosinophilia and systemic symptoms, acute generalised exanthematous pustulosis, erythema multiforme.

CONTRAINDICATIONS

Patients hypersensitive to griseofulvin and patients with hepatic dysfunction should not receive griseofulvin. It may also precipitate or exacerbate lupus in patients with lupus erythematosus or lupus-like syndrome and porphyria in porphyric patients. Griseofulvin is also contraindicated in pregnancy. Some literature warn that men receiving griseofulvin should not father children within six months of treatment. The warning is based on data from in vitro studies using mammalian cells which demonstrated aneuploidy.

WARNING & PRECAUTION

Griseofulvin may impair the ability to drive or operate machinery. Patient monitoring may be warranted in some patients. Complete blood count, serum creatinine concentration, urinalysis and hepatic function determinations are recommended at periodic intervals during therapy.

Severe cutaneous adverse reactions (e.g. Stevens-Johnson syndrome, toxic epidermal necrolysis, drug reaction with eosinophilia and systemic symptoms, acute generalised exanthematous pustulosis) and erythema multiforme have been reported with griseofulvin use. These reactions may be serious and may result in hospitalisation or death. If severe skin reactions occur, griseofulvin should be discontinued.

DRUG INTERACTIONS

The effects of alcohol may be potentiated by griseofulvin. Also, concurrent use with griseofulvin may result in tachycardia, diaphoresis and flushing. Anticoagulant effects may be decreased when used concurrently with griseofulvin due to accelerated metabolism of anticoagulants secondary to stimulation of hepatic microsomal enzyme activity. Prothrombin time should be monitored until a stable level is maintained; dosage adjustments may be necessary during and after griseofulvin therapy. Concurrent long-term use of griseofulvin may decrease the effectiveness of oral contraceptives, resulting in decreased



serum estrogen concentrations; this may lead to intermenstrual bleeding, amenorrhoea, or unplanned pregnancies; patients should be advised to use an alternate or additional method of contraception while taking griseofulvin concurrently with estrogen-containing oral contraceptives and for 1 month after stopping griseofulvin. Antifungal effects of griseofulvin may be decreased when it is used concurrently with primidone or barbiturates, especially phenobarbital, because of impaired absorption, resulting in decreased serum concentrations. Although the effect of decreased serum concentrations on therapeutic response has not been established, concurrent use is preferably avoided.

OVERDOSAGE

The most frequent side effect is headache which is usually mild and transient. Confusion, hypersensitivity (skin rash, hives or itching), oral thrush (soreness or irritation of mouth or tongue), photosensitivity (increased sensitivity of skin to sunlight), dizziness, gastrointestinal reactions (diarrhoea, nausea or vomiting, stomach pain), insomnia and unusual tiredness have been reported less frequently. Dry mouth, altered sensation of taste, have also occurred. Rarely, granulocytopenia or leukopenia (sore throat and fever), angioedema, erythema multiforme, toxic epidermal necrolysis, proteinuria, hepatitis (yellow eyes or skin) and peripheral neuritis (numbness, tingling, pain or weakness in hands or feet) have also occurred.

Up-to-date information on the treatment of overdose can be obtained from The National Poison Center, University Sains Malaysia.

TOXICOLOGY

Cross-sensitivity and/or related problems
Since griseofulvin is derived from a species of Penicillium, it is theoretically possible that patients intolerant of penicillins or penicillamine may be intolerant of griseofulvin also. However, cross-sensitivity between griseofulvin and penicillins or penicillamine has not been clinically substantiated. In addition, penicillin-sensitive patients have received griseofulvin without difficulty.

Carcinogenicity/Tumorigenicity/Mutagenicity
Griseofulvin has been shown to cause hepatomas in several strains of mice, particularly males, that were chronically fed griseofulvin at levels ranging from 0.5%-2.5% of their diet. Smaller prarticle size griseofulvin resulted in an enhanced tumorigenic effect.

Griseofulvin given subcutaneously in relatively small doses once a week during the first 3 weeks of life, has also been shown to cause hepatomas in mice. Griseofulvin has been shown to cause thyroid tumours, mostly adenomas but also some carcinomas, in male rats that were fed griseofulvin at level of 0.2%, 1% and 2% of their diet. Thyroid tumors were also reported in female rats that were fed the two higher dosage level of griseofulvin. Studies in other animal species have not shown that griseofulvin is tumorigenic, however griseofulvin has been shown to have a colchicine-like effect on mitosis and to be cocarcinogenic with metylcholanthrene in cutaneous tumour induction studies in laboratory animals.

Pregnancy/Reproduction

Fertility
Griseofulvin has been shown to suppress spermatogenesis in rats, although this has not been confirmed in humans.

Pregnancy
Griseofulvin crosses the placenta. Conjoined twins have been reported rarely in patients taking griseofulvin during the first trimester of pregnancy. Therefore, this medication is not recommended for use

during pregnancy. Studies in rats have shown that griseofulvin is embryotoxic and teratogenic. In addition, studies in dogs have shown that griseofulvin may cause adverse effects in pups.

Breast-feeding
It is not known whether griseofulvin is excreted in breast milk. However, problems in humans have not been documented.

Pediatrics
Appropriate studies on the relationship of age to the effects of griseofulvin have not been performed in children up to 2 years of age.

Geriatric
Appropriate studies on the relationship of age to the effects of griseofulvin have not been performed in the geriatric population. However, no geriatrics-specific problems have been documented to date.

Dental
Griseofulvin may cause oral thrush (soreness or irritation of mouth or tongue).

Route of administration: Oral

PRESENTATION

Krisovin Tablet 500mg: 10 x 10's, 50 x 10's & 100 x 10's.
All pack size(s) stated may not be available locally

STORAGE CONDITIONS AND USER INSTRUCTIONS

Store in a dry place below 30°C.
Protect from light.
Keep out of reach of children.
Jauhi daripada kanak-kanak.
Shelf life: please refer to outer package.
Take at regular intervals.
Complete the prescribe course unless otherwise directed.
Take medicine with or after meals, especially fatty meals.
May cause dizziness.
Avoid alcoholic beverages.
Avoid to much sun or use of sunlamp.

Product Registration Holder:
DUOPHARMA MANUFACTURING (BANGI) SDN. BHD.
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